

Surgery for melanoma

Most people diagnosed with melanoma will have further surgery after their mole has been removed.

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About surgery for melanoma

Surgery is usually the main treatment for [melanoma](#) that has not spread. It may be the only treatment you need. The aim is to remove the cancer cells completely and the surrounding area of healthy tissue. This is called wide local excision surgery (WLE).

You may have a wide local excision:

- If you had an abnormal mole removed (an [excision biopsy](#)) and this showed it was a melanoma. The aim is to reduce the risk of melanoma coming back.
- To remove [melanoma that has come back in the same area](#).

Surgery for advanced melanoma

The information on this page is about surgery for melanoma that has not spread. Sometimes surgery is used to treat melanoma that has spread to other parts of the body. We have separate information about [advanced melanoma](#).

Having a wide local excision (WLE)

Before the operation, your doctor will talk to you about how much tissue needs to be removed. The amount that is taken depends on how thick the melanoma was.

You usually have a WLE under a local anaesthetic, in a day surgery unit. This means you are awake, but you do not feel the surgery. You will have an injection to numb the area where the operation is done.

Sometimes a WLE is done under a general anaesthetic. This means you are asleep during the operation.

The doctor carefully removes the area of tissue. Depending on the size of the wound and where it is, they may:

- leave the area to heal naturally
- close the wound with stitches
- cover the wound with a skin graft
- cover the wound with a skin flap.

A skin graft is a layer of skin that is taken from another part of the body and placed over the area where the melanoma was removed.

A skin flap is a slightly thicker layer of skin than a graft. It is taken from an area very close to where the melanoma was removed.

Checking the lymph nodes

You may have a test called a [sentinel lymph node biopsy](#) at the same time as a WLE. This depends on the thickness of the melanoma. The test is to check the [lymph nodes](#) for cancer cells.

After a WLE

After surgery, your doctor or nurse will explain how to look after the wound. They will also tell you who to contact if you have wound problems such as:

- bleeding that will not stop
- signs of infection including wound redness, pain, heat or pus.

You can usually go home soon after the operation. You may need to stay longer or overnight if you have a general anaesthetic and sometimes after skin graft surgery. You may need to stay for a few days after skin flap surgery. Your doctor or nurse can give you more information.

Wounds

Your wound may look red and swollen at first, but this will gradually get better. You will be given painkillers to help after the operation.

Your nurse will explain if you have stitches that need to be removed. Your nurse may also give you dressings to protect the wound while it heals. They will explain when and how to change your dressing.

If you had a skin graft, you will also have a wound where the skin was taken from. This is called the donor site. How long this area takes to heal depends on how much skin was removed.

Try not to do too much in the first 2 weeks after skin graft surgery. It is important not to put pressure on the grafted area such as rubbing or brushing against it. The skin graft takes about 5 to 7 days to connect with the blood supply in the area.

Scar

Removing tissue during a WLE will always leave a mark on the skin. Before your operation, your doctor will explain what to expect. Your scar after melanoma removal depends on the location and amount of tissue that was removed.

If you had a skin graft, you will have a scar in both the grafted and the donor site area. There will also be some differences between the grafted skin and the skin surrounding it. But this will become less noticeable over time.

If scars after surgery bother you or make you [feel self-conscious](#), our information about [body image and cancer](#) may help. Some skin clinics have a make-up specialist who can give you advice on the best way to cover up scars.

Further tests and treatment

If all the melanoma cells are removed during your wide local excision, you will not need any more treatment.

[Tests to check the lymph nodes](#) may show melanoma cells in the lymph nodes. In this situation you may be offered further [surgery to remove all the nearby lymph nodes](#). Your cancer doctor will talk to you about whether this may be helpful in your situation.

If the melanoma has spread to the lymph nodes, you might be offered further treatment with [targeted therapy](#) or [immunotherapy](#).

Depending on your surgery, you may have some small scars or areas of skin that look different. Changes to your appearance may make you worried about your [body image](#). Some skin clinics have a make-up specialist who can give you advice on the best way to cover up scars.

We have [more information about follow-up after treatment for melanoma and recurrent melanoma](#).

Follow-up

After surgery you will usually have a follow-up appointment with your doctor or nurse. They will check your wound and explain if you need any further treatment. They will also give you information about [checking your skin](#) and [how to stay safe in the sun](#).

If you had a [melanoma in situ \(stage 0\)](#) you will not need any further appointments. If you have further questions or are worried about new symptoms, your GP can help. If needed they will arrange for you to see a specialist again.

If you had a thicker [melanoma that has not spread \(stage 1 or 2\)](#) you may have ongoing follow up appointments. Some people will also have a regular scans or ultrasounds of the nearby lymph nodes. Your doctor or nurse will explain what to expect.

About our information

References



Below is a sample of the sources used in our melanoma information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Michielin O, van Akkooi ACJ, Ascierto PA, et al. Cutaneous melanoma: ESMO Clinical Practice Guidelines for diagnosis, treatment and follow-up. *Annals of Oncology*. 2019; 30, 12, 1884-1901 [accessed May 2022].

Michielin O, van Akkooi ACJ, Ascierto PA, et al. ESMO consensus conference recommendations on the management of locoregional melanoma: under the auspices of the ESMO Guidelines Committee. *Annals of Oncology*. 2020; 31, 11, 1449-1461 [accessed May 2022].

Peach H, Board R, Cook M, et al. Current role of sentinel lymph node biopsy in the management of cutaneous melanoma: A UK consensus statement. *Journal of Plastic, Reconstructive & Aesthetic Surgery*. 2020; 73, 1, 36-42 [accessed May 2022].

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Professor Samra Turajlic, Consultant Medical Oncologist.

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Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 October 2022 | Next review: 01 October 2025

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help